



BREAST FEEDING



Nourish. Protect. Bond. Breastfeeding is the best start for your baby.

PERFECT NUTRITION



Breast milk contains all the nutrients your baby needs for healthy growth and development.

BOOSTS IMMUNITY



Antibodies in breast milk help protect your baby from infections and diseases.

STRENGTHENS BONDING



Breastfeeding builds a strong emotional bond and sense of security for your baby.

SUPPORTS BRAIN DEVELOPMENT



Essential nutrients in breast milk support brain growth and cognitive development.

CONVENIENT & ALWAYS READY



Always clean, the right temperature, and readily available anytime, anywhere.

PROMOTES HEALTHY WEIGHT



Helps maintain a healthy weight for your baby and reduces the risk of obesity later in life.

BENEFITS MOM



Helps uterus recovery, reduces risk of certain cancers, and supports healthy weight management.

COST-FREE & ECO-FRIENDLY



Breast milk is free, reduces medical costs, and is environmentally friendly.

FEED ON DEMAND



Feed your baby whenever they are hungry—day or night. It helps build your milk supply.

EXCLUSIVE FOR FIRST 6 MONTHS



Give only breast milk for the first 6 months for the best health outcomes.



Every drop of love. Every feed counts.

Support. Nourish. Thrive.

Topic: 4



BREASTFEEDING

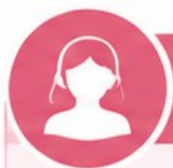


Exclusive breastfeeding should be given for the first six months of life; complimentary food should be started after six months of age.



BENEFITS TO THE BABY

- Complete food, species specific
- Easily digested and well absorbed
- Protects against infection
- Promotes emotional bonding
- Better brain growth



BENEFITS TO MOTHER

- Helps in involution of uterus
- Delays pregnancy
- Lowers risk of breast and ovarian cancer
- Decreases mother's work load



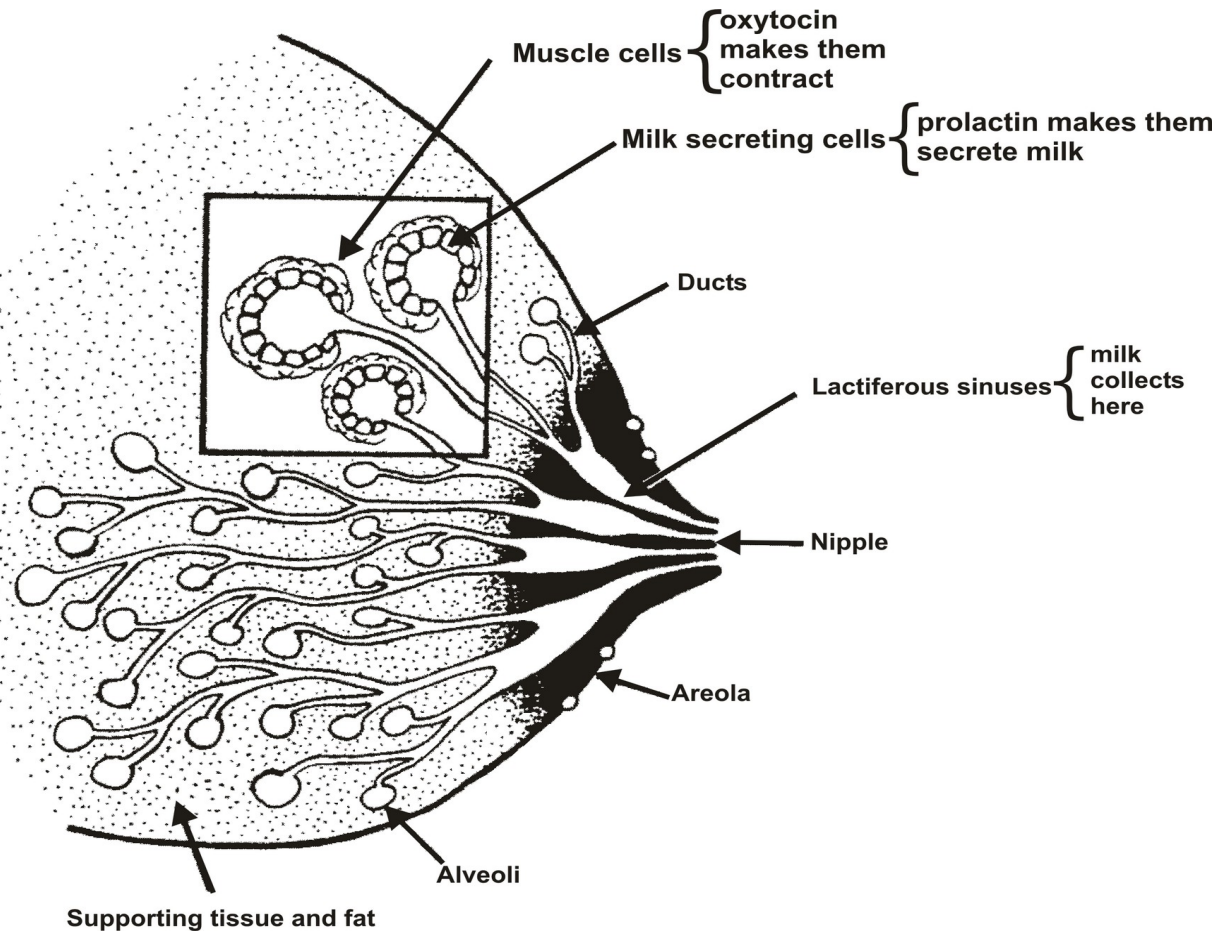
BENEFITS TO FAMILY AND SOCIETY

- Saves money
- Promotes family planning
- Decreases need for hospitalization
- Contributes to child survival

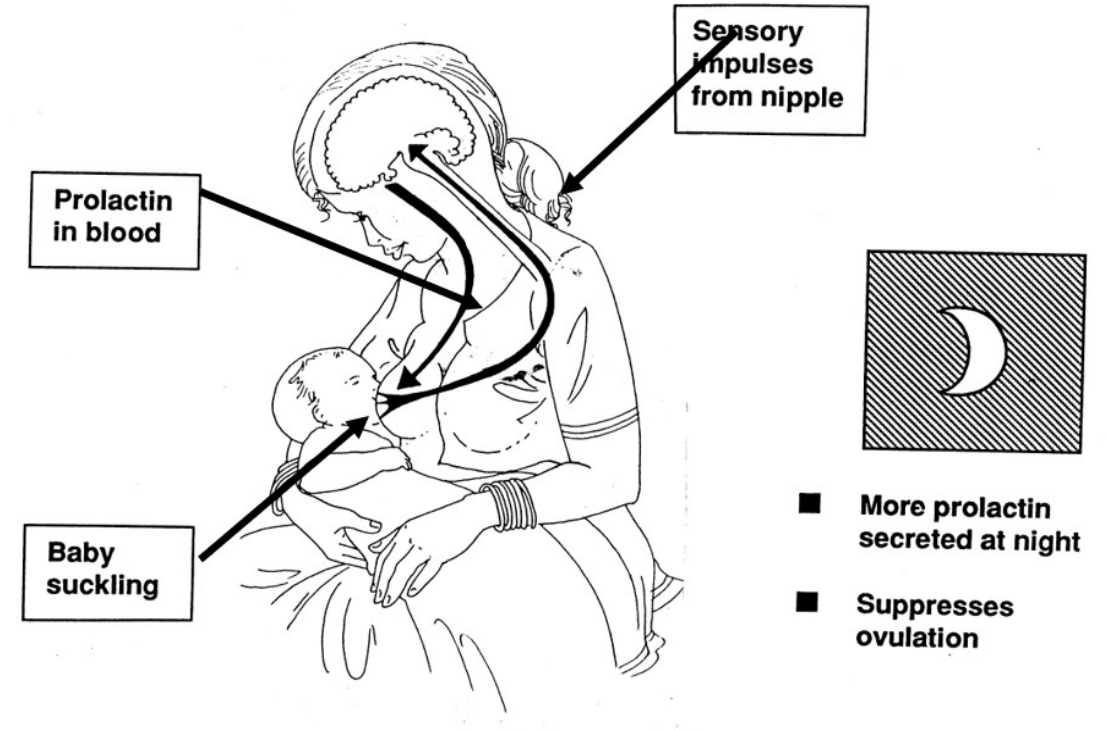


Breastfeeding is the best start for every baby and the best choice for every family.

Breast Anatomy - Structure



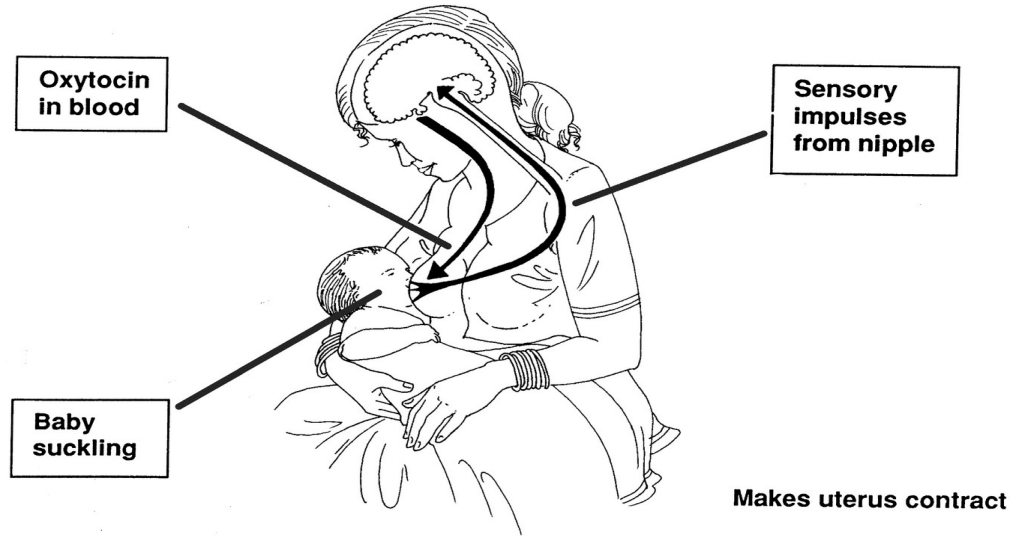
Prolactin Reflex



To increase milk productions

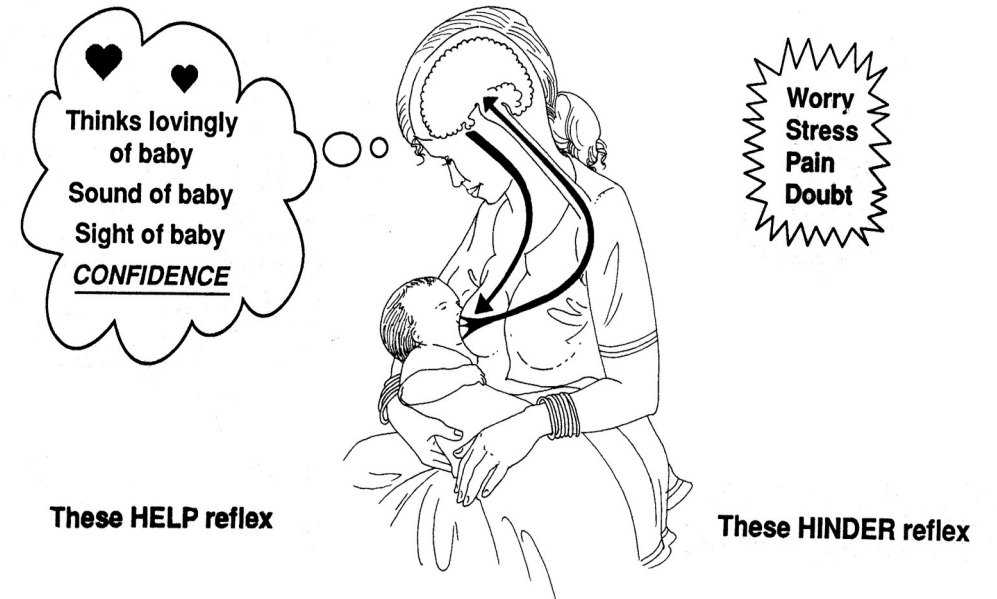
Oxytocin Reflex

Works *BEFORE* or *DURING* feed
to make milk *FLOW*



For milk ejection

Helping and Hindering the Oxytocin Reflex



BREASTFEEDING



1. PREPARING THE INFANT AND THE MOTHER



Ensure that the infant is clinically stable.



Ensure that the infant is alert.



Make sure that the mother is comfortable and relaxed.



She should sit down in a comfortable and convenient position.



2. FOUR KEY POINTS IN POSITION

1

The baby's head and body should be straight.



2

The baby's face should face mother's breast.



3

The baby's body should be close to her body.



4

She should support the baby's whole body.



3. HOW TO SUPPORT BREAST WITH THE OTHER HAND



Explain the mother that she should.

Put her fingers below her breast (keep index finger).

Put her thumb above the areola, helping to shape the breast.

Not to keep her fingers near the nipple.

BREASTFEEDING



HOW TO HELP THE BABY TO ATTACH



1

Ask the mother to express a little milk onto her nipple.



2

Touch the baby's lips with her nipple.



3

Wait until the baby's mouth is opening wide, and the tongue is down and forward.



4

Move the baby quickly onto her breast, aiming the nipple towards the baby's palate and his lower lip well below the nipple.

SIGNS OF GOOD ATTACHMENT



More areola is visible above the baby's mouth than below it.



The baby's mouth is wide open.



The baby's lower lip is turned outwards.



The baby's chin is touching the breast.

THE CAUSES OF POOR ATTACHMENT



Use of feeding bottles.



Inexperienced mother.



Lack of skilled support.



Inverted nipples.



CONSEQUENCES OF POOR ATTACHMENT



Pain and damage to nipples

- Sore nipples
- Fissures



Breastmilk not removed effectively

Engorgement



Apparent poor milk supply

Baby unsatisfied, wants to feed more

Baby frustrated, refuses to suckle



Breasts make less milk

Baby fails to gain weight



Good attachment helps the baby get enough milk and keeps both mother and baby healthy and happy.

Assess if the infant is suckling and swallowing effectively

Effective suckling



Infant takes several slow deep sucks followed by swallowing, and then pauses.

Ineffective suckling



Infant suckles for a short time, but tires out and is unable to continue for long enough

How frequently a mother has to breastfeed her baby?



A healthy newborn baby can be breastfed **ON DEMAND**.



The usual time interval between each feed is about **2 to 3 hours**.



Mothers should be advised that they should feed their babies **AT LEAST 8–10 times** in 24 hours and importantly they should not omit any night feeds.



Assessing the adequacy of breastfeeding



Passes urine 6–8 times in 24 hours.



Goes to sleep for 2–3 hrs after the feeds.



Gains weight @10–15 gm/kg/day.



Crosses birth weight by 2 weeks.

HUNGER CUES OF NEWBORN

EARLY HUNGER CUES

(MILD)



- Sucking on fingers, hands, or nearby objects
- Smacking or licking lips
- Rooting reflex: turning their head toward the breast or bottle when their cheek is touched
- Fidgeting or small movements

MID HUNGER CUES

(MORE ACTIVE)



- Increased movement or squirming
- Trying to position themselves for nursing
- Fussing or mild agitation

LATE HUNGER CUES

(CALMING NEEDED FIRST)



- Crying
- Agitated movements
- Red in the face
- Arching of the back
- Difficulty latching on

VERY LATE HUNGER CUES

(CALMING NEEDED FIRST)



- High-pitched crying
- Extreme agitation
- Erratic body movements
- Pushes away
- Difficult to console and latch



Responding to early hunger cues helps your baby feed better and stay calm.

ISSUES IN BREASTFEEDING



**INVERTED /
FLAT NIPPLES**



SORE NIPPLES



**BREAST
ENGORGEMENT**



**BREAST
ABSCESS**



NOT ENOUGH MILK

INVERTED NIPPLE – MANAGEMENT

- 1. NIPPLE STIMULATION:** Gently roll or pinch the areola to encourage the nipple to protrude.
- 2. SUCTION DEVICE:** Use a breast pump before feeding to draw out the nipple.
- 3. NIPPLE SHIELD:** Use a nipple shield if needed, under guidance, to aid effective latch.
- 4. REGULAR FEEDING:** Frequent breastfeeding helps the nipple become more everted over time.

• SORE NIPPLE – CAUSES & TREATMENT •

SORE NIPPLE



CAUSES

- Incorrect attachment: Nipple sucking
- Frequent use of soap and water
- Fungal infection of nipple

TREATMENT

- Continue breast feeding and change position.
- Attach baby to the areola while feeding.
- Apply hind milk to the nipple after breast feed.
- Expose the nipple to air between feeds. Do not wash each time before and after feed.
- If fungal infection apply medicine on the nipple and inside the mouth of the baby.

TIPS

- Ensure proper latch in every feed.
- Use plain water for cleaning once daily.
- Wear a comfortable, breathable bra.
- Seek lactation support if pain persists.



BREAST ENGORGEMENT



CAUSES

- Plenty of milk
- Delay starting to breastfeed
- Poor attachment to breast
- Infrequent removal of milk
- Restriction of length of feeds



PREVENTION

- Start breastfeeding soon after delivery
- Ensure good attachment
- Encourage unrestricted breastfeeding



TREATMENT

- Local warm water packs for not more than 15 minutes. Paracetamol can be given to the mother to relieve pain.

BREAST ABSCESS



The mother may have high grade fever and pain in breast



TREATMENT: Mother must be treated with analgesics and antibiotics.



The abscess must be incised and drained.



Breast feeding must be continued from the other breast.

NOT ENOUGH MILK



CAUSES

- Not breastfeeding frequently
- Too short or hurried breastfeeds
- Poor position
- Breast engorgement or mastitis



TREATMENT

- More frequent feeds, especially at night
- Proper attachment to breast
- Care of painful condition
- Back massage for stimulating lactation

